

# Pre-Collection Revenue Cycle Optimization

Dental and Outpatient Chains • CFO / Ownership Briefing

---

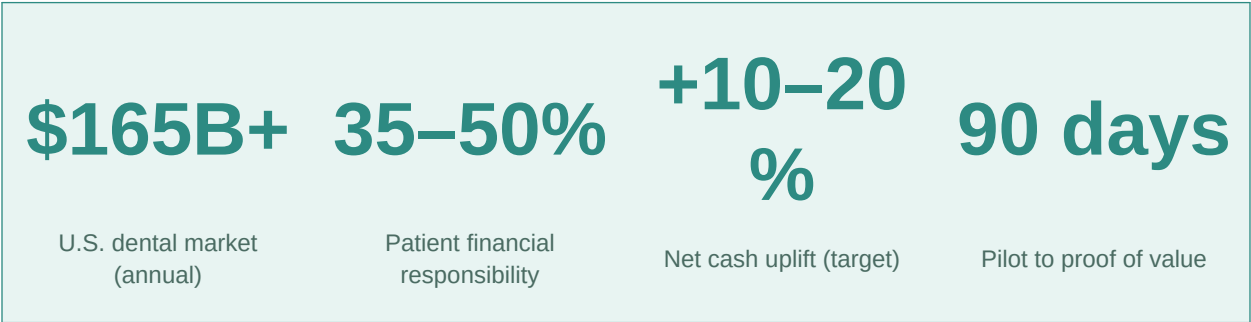
**CONFIDENTIAL INSTITUTIONAL DECK**

VitaCoreX LLC • Tampa, FL • EIN 41-4399148  
(888) 794-8292 • vitacorexllc.com

© 2026 VitaCoreX LLC. All rights reserved. Confidential.

# Executive Summary

Patient accounts receivable represents one of the largest controllable cash drivers in multi-location dental groups. With patient responsibility comprising 35–50% of dental revenue and recovery probability decaying sharply with aging, the economic case for structured pre-collection infrastructure is substantial. This deck presents the market context, financial models, operating system design, and pilot economics for implementing a pre-collection recovery layer in dental and outpatient environments.



Sources: ADA Health Policy Institute 2024; IBISWorld U.S. Dental Services 2024; ADSO Financial Benchmarking 2024.

# U.S. Dental Market Overview

The U.S. dental services market generates over \$165 billion in annual revenue across approximately 200,000 dental practices. Dental service organizations (DSOs) now support an estimated 25–30% of practicing dentists and are growing rapidly through acquisition and de novo expansion. This consolidation creates both the need and the opportunity for standardized revenue cycle infrastructure.

| Metric                       | Value                  | Source                |
|------------------------------|------------------------|-----------------------|
| U.S. dental market size      | \$165B+ annually       | ADA HPI 2024          |
| Total dental practices       | ~200,000               | ADA 2024              |
| DSO market penetration       | 25–30% of dentists     | ADSO 2024             |
| Patient responsibility share | 35–50% of revenue      | Industry estimates    |
| Typical AR days (dental)     | 35–60 days             | Dental Economics 2024 |
| Annual DSO M&A; deals        | 200+ transactions/year | Becker's Dental 2024  |

Sources: ADA Health Policy Institute 2024; Association of Dental Support Organizations; Becker's Dental Review 2024; Dental Economics Annual Survey 2024.

## Patient AR: The Least Structured Revenue Segment

Unlike insurance AR — which follows standardized claim/remittance cycles — patient AR depends on individual billing practices, staff communication, and payment plan management. For dental groups, this segment is large (35–50% of revenue) and systematically under-managed.

| AR Category            | Revenue Share | Collection Efficiency  |
|------------------------|---------------|------------------------|
| Insurance pending      | 40–50%        | ~96% (payer-driven)    |
| Patient responsibility | 35–50%        | ~55–65% (staff-driven) |
| Self-pay / uninsured   | 5–15%         | ~25–35% (high risk)    |

# Recovery Probability by Aging

Dental patient balances follow the same exponential decay curve as broader healthcare: every 30-day delay materially reduces recovery probability. The critical intervention window is 0–60 days, where structured outreach, payment plan offers, and autopay enrollment can capture the majority of recoverable cash.

| Aging Bucket | Recovery Probability | Action Required                       |
|--------------|----------------------|---------------------------------------|
| 0–30 days    | 90–95%               | Automated statement + payment portal  |
| 31–60 days   | 70–80%               | Structured outreach + plan offer      |
| 61–90 days   | 45–55%               | Escalated contact + autopay push      |
| 91–120 days  | 25–35%               | Final notice + pre-agency assessment  |
| 120+ days    | 10–18%               | Agency referral or write-off decision |

Source: ACA International Recovery Benchmarking 2024; HFMA AR Aging Analysis 2024.

# Financial Model: Hidden Cash in Patient AR

## Illustrative Model: Mid-Size Dental Group

| Metric                                     | Value                  |
|--------------------------------------------|------------------------|
| Annual revenue                             | \$40,000,000           |
| Patient responsibility (40%)               | \$16,000,000           |
| Average patient AR outstanding             | \$5,200,000            |
| Current recovery rate                      | ~60%                   |
| Target recovery rate (with pre-collection) | ~75%                   |
| Recovery improvement                       | +\$780,000/year        |
| Revenue leakage prevented                  | \$520K–\$780K annually |

## DSO Compression Impact

| Metric                    | Before  | After   | Impact                     |
|---------------------------|---------|---------|----------------------------|
| Days Sales Outstanding    | 52 days | 44 days | -8 days                    |
| Working capital release   | —       | —       | ~\$0.22M per \$10M revenue |
| Cash velocity improvement | —       | —       | Faster reinvestment cycle  |

## EBITDA and Enterprise Value

| Metric                            | Conservative  | Target        |
|-----------------------------------|---------------|---------------|
| Incremental recovered cash        | \$400,000     | \$650,000     |
| EBITDA contribution (high margin) | \$320,000     | \$520,000     |
| Dental/DSO EV multiple            | 7–9x          | 7–9x          |
| Enterprise value impact           | \$2.2M–\$2.9M | \$3.6M–\$4.7M |

Source: PitchBook Dental Services Multiples 2024; ADSO Financial Benchmarking 2024.

## 90-Day Pilot Design

A data-validated pilot proves the model before enterprise rollout. The pilot targets a defined AR cohort across 2–4 locations with measurable success criteria agreed in advance with CFO/ownership.

| Phase          | Timeline   | Scope                          | Deliverable              |
|----------------|------------|--------------------------------|--------------------------|
| Diagnostic     | Weeks 1–2  | AR export + workflow audit     | Gap report + pilot scope |
| Setup          | Weeks 2–4  | Segmentation + scripts + tools | Operational playbook     |
| Execution      | Weeks 4–8  | Outreach + plans + autopay     | Recovery activity log    |
| Optimization   | Weeks 8–12 | Refine cadence + exceptions    | Performance report       |
| Results review | Week 13    | CFO presentation               | Scale/no-scale decision  |

## Pilot Financial Targets

| Metric                    | Value         |
|---------------------------|---------------|
| Patient AR in scope       | \$2.0M        |
| Expected improvement      | +10–15%       |
| Additional cash recovered | \$200K–\$300K |
| Pilot cost                | \$50K         |
| ROI                       | 4–6x          |

# Operating System Architecture

The pre-collection operating system is a tool-agnostic layer that sits on top of existing practice management and billing systems. It standardizes the workflow between billing and agency referral without replacing existing infrastructure.

## Workflow Sequence

| Step            | Action                                           | Documentation Required                      |
|-----------------|--------------------------------------------------|---------------------------------------------|
| 1. Intake       | Capture financial data + consent at registration | Financial agreement, insurance verification |
| 2. Segmentation | Route by balance, age, risk profile              | Segmentation rules, account tags            |
| 3. Outreach     | Standardized multi-channel contact               | Call notes, SMS/email templates             |
| 4. Plan offer   | Structured payment plan with terms               | Signed agreement, payment schedule          |
| 5. Autopay      | ACH/card enrollment at plan execution            | Authorization form, recurring setup         |
| 6. Monitoring   | Exception queue + cure management                | Default notices, dispute records            |
| 7. Escalation   | Documented handoff to counsel/agency             | Complete evidence package                   |

## Governance Model

- Weekly operations review: exception volumes, outreach completion, autopay enrollment
- Monthly finance review: net recovery yield, DSO trends, cohort performance
- Quarterly optimization: script refinement, offer catalog updates, compliance review

# Staff Training and Compliance

## Operational Discipline Without Adding Headcount

- Payment plan negotiation and options-first framing
- Script training: calls, SMS, email with cadence limits
- Compliance boundaries and escalation governance
- Exception handling: disputes, hardship, bad contact data
- CRM workflow: tasks, documentation, audit trail, reporting

## Compliance Framework

- FDCPA/Reg F alignment for pre-collection activities
- HIPAA-compliant billing communications
- Cadence caps and quiet hours enforcement
- Dispute routing and hardship assessment protocols
- Audit trail for all patient financial interactions

## Patient Experience Safeguards

- Options-first messaging: payment plan before collection language
- Brand-safe tone in all communications
- Online reputation monitoring and response protocol



## CFO One-Page Summary

| Area       | Detail                                                                          |
|------------|---------------------------------------------------------------------------------|
| Problem    | Patient AR aging destroys recovery probability and increases agency fee leakage |
| Solution   | Pre-collection operating system: standard workflows + controls + CFO reporting  |
| Impact     | +10–20% net recovery; placement reduction; DSO compression; EBITDA uplift       |
| Evidence   | Industry benchmarks from HFMA, MGMA, ACA International, ADA HPI                 |
| Approach   | 90-day pilot to validate with live data before enterprise rollout               |
| Investment | \$50K pilot cost; 4–6x ROI target; 3–4 month payback                            |

## Next Steps

| Step | Action                                       |
|------|----------------------------------------------|
| 01   | 15-minute fit call to confirm alignment      |
| 02   | NDA + AR export request                      |
| 03   | Diagnostic review + pilot scope confirmation |
| 04   | Pilot launch                                 |

**vitacorexllc.com | (888) 794-8292**

*VitaCoreX LLC provides administrative, documentation, workflow, and business-support services. It is not a law firm and does not provide legal representation.*